

Chapter 16: The AHS Restructuring Roadmap — A Six-Pillar Framework for System Architects

The AHS Restructuring Roadmap — A Six-Pillar Framework for System Architects

Redesigning Vermont's Agency of Human Services for the 2028 Strategic Plan Mandate: A Six-Pillar Framework for System Architects

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (2025); Health Care Delivery Advisory Committee mandate; AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); AHS January 2025 Legislative Presentation; GMCB Act 68 Update (February 2026).

This chapter is written for a specific audience: the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, GMCB officers, and hospital and community provider leaders who must translate Vermont's reform legislation into an operational reality. It is simultaneously academic — grounding every recommendation in evidence and analytical framework — and operational — providing the specific organizational design, governance structure, accountability metrics, and implementation timeline that Vermont's transformation requires. The six-pillar framework that organizes this entire book maps directly onto the Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont Legislature by December 2028\.

The Statutory Mandate — What Act 68 Actually Requires AHS to Do

The most important thing to understand about Act 68's mandate to AHS is that it is not a mandate to write a plan. It is a mandate to restructure a system. The Statewide Health Care Delivery Strategic Plan is the vehicle, not the destination. The destination is a Vermont healthcare system that achieves the five goals of Act 167: reduced inefficiencies, lower costs, improved health outcomes, reduced health inequities, and increased access to essential services. The plan is the document that makes AHS accountable for moving toward those goals.

Act 68 is specific about what the Strategic Plan must contain. Drawing from the statute and the Health Care Delivery Advisory Committee mandate, the plan must address:

Statutory requirement	What it means in practice	Deadline
Statewide Health Care Delivery Strategic Plan	A comprehensive plan for Vermont's healthcare delivery system covering all six dimensions: payment reform trajectory, primary care investment targets, hospital network configuration, behavioral health integration, equity goals, and administrative simplification. Updated every three years.	First delivery: December 1, 2028

Statutory requirement	What it means in practice	Deadline
Affordability benchmarks	Specific, measurable standards for health care affordability — not aspirational language. Must define what 'affordable' means for Vermont families and how progress toward that standard will be measured.	Established by Health Care Delivery Advisory Committee; incorporated in Strategic Plan
Total cost of care targets	Quantified targets for per-capita healthcare cost growth, aligned with AHEAD Model commitments and Act 68's hospital spending reduction requirements.	Act 68 requires 2.5% hospital spending reduction for FY2026; AHEAD TCOC targets from 2027
Standardized accountability metrics	A defined set of outcome measures — tied to the five Act 167 goals — against which AHS will report progress to the legislature monthly and the Strategic Plan will be evaluated.	AHS already reporting monthly; full metrics framework needed by Strategic Plan delivery
Primary care investment plan	A plan for increasing primary care investment as a percentage of total healthcare spending — an explicit AHEAD requirement — including Blueprint expansion, CCBHC development, and CoCM deployment.	AHEAD requires primary care investment targets from 2027; Strategic Plan formalizes the plan
Health equity strategy	A specific plan for reducing disparities in access and outcomes resulting from demographic factors or health status — required explicitly by Act 68 Goal 1\.	Embedded in Strategic Plan; equity measures in monthly AHS reporting
Workforce strategy	A plan for attracting and retaining high-quality healthcare professionals in Vermont — Act 68 Goal 5 — including the use of RHT Program workforce investments.	RHT Program plan covers 2026–2030; must be integrated into Strategic Plan

Figure 16.1 — Act 68 statutory requirements for the Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate (healthcarereform.vermont.gov).

The Strategic Plan is not AHS's alone to write. Act 68 created the Health Care Delivery Advisory Committee — 18 members representing AHS, GMCB, the Health Care Advocate, hospitals, health centers, insurers, health professions, small businesses, and long-term care facilities — to develop and maintain the plan in collaboration with AHS. The committee also consults with the Health Equity Advisory Commission and the Vermont Steering Committee for Comprehensive Primary Health Care. This governance structure means the Strategic Plan must reflect multi-stakeholder input and consensus, not just AHS's internal analysis — a significant operational challenge and an important political constraint.

"Review existing AHS agency structure and program list to identify overlaps and opportunities for efficiency. Align Agency of Human Services agencies with Health Service Areas and meet regularly with hospitals and providers. Fully computerize and integrate Agency of Human Services subunits."

The Organizational Diagnosis — Why AHS Must Restructure, Not Just Plan

Oliver Wyman's finding about AHS was specific and unambiguous. After 230 meetings with hospital leaders, community organizations, state agency staff, and Vermonters across all 14 Hospital Service Areas, the consultants identified AHS's organizational structure as one of the primary barriers to effective healthcare transformation — not because AHS lacks capable people or good intentions, but because the organization was designed for a different era and a different role.

The current AHS organizational model is what management theorists call a program-silo structure: it is organized around funding streams and program types (Medicaid, mental health, substance use, aging services, developmental services, Blueprint, health care reform) rather than around the populations it serves or the geographic communities those populations live in. This structure made sense when AHS's primary function was administering categorical benefit programs. It does not make sense for an organization whose primary function has become managing the transformation of an interconnected healthcare delivery system.

The Four Structural Gaps

Oliver Wyman identified four specific structural gaps that AHS's transformation must address. These are not recommendations for improvement — they are diagnoses of organizational failures that are currently impeding transformation.

Gap	What the gap looks like in practice	Consequence for transformation	Required structural change
Program-silo structure misaligned with Hospital Service Areas	A hospital in the Northeast Kingdom that needs to address housing, transportation, behavioral health, and primary care gaps simultaneously must work with separate AHS programs: DVHA for Medicaid, DMH for mental health, ADS for aging services, DAIL for developmental services, Office of Health Care Reform for transformation planning. Each has different administrative requirements, reporting cycles, and funding authorities.	Hospitals and community providers experience AHS as a collection of disconnected programs rather than a coordinated partner. Population health management — which requires crossing program boundaries — is administratively complex to the point of being effectively impossible.	Designate HSA-level coordinators within AHS who convene all relevant AHS programs around each hospital's service area; establish HSA-level governance structures with regular inter-program coordination
Absence of a Planning and Effectiveness function	AHS currently has no dedicated analytical unit that can model the financial and access impact of proposed service site changes, calculate the system-wide cost implications of different regionalization scenarios, or monitor whether transformation investments are	AHS cannot answer the questions the transformation planning process is supposed to generate: if Springfield Hospital converts to REH status, what is the net impact on healthcare access and system cost in Windsor County? If primary care investment increases by \$X, what is the	Establish a Division of Planning, Analytics, and Effectiveness within AHS; partner with GMCB on a joint analytics platform; integrate VHCURES access for AHS planning functions

Gap	What the gap looks like in practice	Consequence for transformation	Required structural change
	producing the intended outcomes.	projected reduction in avoidable ED visits? Without this analytical capacity, strategic planning is opinion-based, not evidence-based.	
Under-resourcing for the transformation management function	Act 68 requires AHS to simultaneously: negotiate AHEAD implementation with CMS; manage 14 hospital transformation planning processes; develop the Statewide Strategic Plan; oversee \$195M RHT Program implementation; coordinate with GMCB on RBP and global budget design; and report monthly to the legislature. The Health Care Reform Office has neither the staff nor the organizational authority to manage this portfolio effectively.	Transformation activities are sequenced by capacity rather than by strategic priority. The RHT Program contract management alone requires a project manager and contract oversight capacity that does not currently exist. The absence of adequate project management infrastructure is the single most likely cause of transformation plan failure.	Hire dedicated transformation staff using RHT Program implementation funds (explicitly authorized); establish a Project Management Office within the Health Care Reform Office; clarify reporting lines and authority between Health Care Reform and program divisions
Fragmented SDOH integration	AHS has programmatic authority over housing supports, food assistance, transportation programs, mental health, substance use, and developmental services — all of the social determinants that Oliver Wyman's systems map showed driving healthcare utilization. But these programs are administered separately, reported separately, and evaluated separately from healthcare outcomes.	Vermont cannot achieve the SDOH-driven utilization reduction that global budgets require if the programs that address SDOH needs are organizationally disconnected from the healthcare delivery system. Every hospital transformation plan that identifies housing or transportation as a utilization driver requires AHS to coordinate across at least three separate program areas to address it.	Establish cross-program SDOH integration protocols; create a shared data architecture that links social service utilization to healthcare outcomes in VHCURES; include SDOH program directors in HSA-level coordination structures

Figure 16.2 — AHS organizational gaps and required structural changes. Sources: Oliver Wyman Act 167 Recommendations (2024); AHS Transformation Reports (2025); Act 68 of 2025\.

The Redesigned AHS Operating Model — A Population Health System Operator

The organizational model that AHS must become — and that the Statewide Strategic Plan must articulate — is a Population Health System Operator: an agency that manages Vermont's healthcare and social services ecosystem as an

integrated system oriented around population health outcomes, organized geographically around Hospital Service Areas, and accountable for measurable progress toward the five Act 167 goals.

This is a fundamentally different organizational identity than AHS has historically held. The shift is not cosmetic — it requires changes to organizational structure, staff roles, governance processes, data systems, external relationships, and the agency's own theory of how it creates value. The following sections develop this redesigned model across the three levels at which AHS must operate: statewide system architect, regional network convener, and program delivery operator.

Level 1 — Statewide System Architect

At the statewide level, AHS functions as the architect of Vermont's healthcare delivery system: setting the strategic direction, establishing the payment reform trajectory in partnership with GMCB, allocating capital investments (RHT Program, AHEAD EAST Fund, Act 68 grants), developing the accountability framework, and monitoring system performance against the five Act 167 goals.

The statewide architecture function requires four specific organizational capacities that AHS is currently building but has not yet fully developed:

- **Strategic planning capacity:** The ability to develop, maintain, and update the Statewide Health Care Delivery Strategic Plan on a three-year cycle, incorporating input from the Health Care Delivery Advisory Committee, the Steering Committee for Comprehensive Primary Health Care, the Health Equity Advisory Commission, and other required stakeholders. This is a substantial ongoing function, not a one-time project.
- **Analytics and modeling capacity:** The ability to model the cost, quality, and access implications of proposed system changes before they are implemented — answering questions like 'what happens to access in Windsor County if Springfield Hospital converts to REH status?' and 'what is the projected PMPM cost reduction from the primary care investment plan?' This requires the analytics vendor being procured jointly with GMCB, plus internal staff who can interpret and apply the outputs.
- **Federal relationship management:** Active management of Vermont's relationships with CMS (AHEAD Model), HRSA (RHT Program), CMMI (transformation grants), and other federal partners. These relationships are high-stakes, technically complex, and require dedicated staff who understand both Vermont's specific circumstances and the federal program requirements. The AHEAD State Agreement alone requires ongoing negotiation of methodology, monitoring, and potential withdrawal conditions.
- **Legislative accountability:** The production and delivery of the required monthly transformation reports, the December 2028 Strategic Plan, and the three-year updates. Vermont's legislature has demonstrated active engagement with healthcare transformation — the monthly reporting requirement is a real accountability mechanism, not a formality. AHS must staff this function appropriately.

The Health Care Delivery Advisory Committee: AHS's Governance Partner

Act 68 established an 18-member Health Care Delivery Advisory Committee to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Understanding this committee's composition and mandate is essential for AHS's operational planning, because the committee is not an advisory body in the traditional sense — it has statutory authority to set affordability benchmarks and advise the legislature directly.

Committee composition (18 members):

- AHS Secretary (or designee)
- GMCB Chair (or designee)
- Office of the Health Care Advocate
- Representatives from hospitals, health centers (FQHCs), health insurers
- Representatives from nursing, pharmacy, primary care, and other health professions
- Small business representative
- Long-term care facility representative
- Additional members as specified

Primary responsibilities:

- Setting affordability benchmarks — defining measurable standards for what 'affordable healthcare' means for Vermont families
- Monitoring Vermont's healthcare system performance and its impact on public health
- Collaborating with AHS to develop and maintain the Statewide Strategic Plan
- Incorporating recommendations from the Steering Committee for Comprehensive Primary Health Care
- Advising GMCB on creating a continuous evaluation process for the delivery system

- Providing coordinated recommendations to the General Assembly on healthcare delivery

Operational implication for AHS: The committee requires dedicated staff support — meeting facilitation, research preparation, deliberation management, and recommendation documentation. AHS should treat committee support as a dedicated function, not an add-on to existing Health Care Reform Office responsibilities.

Level 2 — Regional Network Convener (HSA-Level)

At the Hospital Service Area level, AHS must function as a regional network convener — bringing together the hospitals, primary care practices, designated mental health agencies, FQHCs, community organizations, and social service providers within each HSA to develop and execute a coordinated regional transformation strategy.

This is where Oliver Wyman's most important organizational recommendation applies: align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers. The mechanism for doing this is the designation of HSA-level AHS coordinators — staff whose primary function is to be the face of AHS within a defined geographic area, convening the relevant AHS programs and community partners around the hospital service area's specific transformation priorities.

Vermont has 14 Hospital Service Areas and a geographically distributed health system. The Blueprint for Health already deploys field staff in each HSA — its Quality Improvement Facilitators, Community Health Team coordinators, and field service staff are embedded in local communities. The Blueprint's HSA infrastructure is the organizational model AHS should replicate and build on for broader transformation coordination.

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
Burlington	UVM Medical Center	\~230K, growing, most diverse; Chittenden County non-rural by RHT definition	Administrative overhead reduction at UVMMC; primary care access for growing immigrant population; behavioral health integration at scale
Barre/Montpelier	Central VT Medical Center	\~65K, aging, state government center	Radiation therapy and neurology COE development; state employee health plan RBP transition; rural access in Washington County
Rutland	Rutland Regional Medical Center	\~62K, aging, lower income	COE development (surgery, neurology, psych); high opioid overdose rates; workforce housing shortage
Burlington-south (White River Jct.)	Mt. Ascutney Hospital	\~59K, aging, Dartmouth Health connection	REH or focused scope transition; Dartmouth Health cross-border coordination; rehabilitation COE
St. Albans	NW Medical Center	\~53K, rural, Franklin/Grand Isle counties	Cancer surgery and radiation COEs; workforce recruitment; Northern border community needs

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
Bennington	SW VT Medical Center	\~48K, aging, most rural	9 COEs — secondary RSC role; psych services critical; Berkshire/Taconic regional coordination
Middlebury	Porter Medical Center	\~46K, aging, Addison County	TBD tier status; Porter affiliated with UVM network; shared service opportunities with UVMMC
Brattleboro	Brattleboro Memorial Hospital	\~42K, aging, high SUD/MH burden	Acute general surgery COE; Windham County high suicide/SH rates; Brattleboro Retreat psychiatric coordination
Newport	North Country Hospital	\~41K, very rural, Northeast Kingdom	One of most financially vulnerable; REH candidacy assessment; Essex/Orleans county high uninsurance
St. Johnsbury	NE VT Regional Hospital	\~39K, very rural, Northeast Kingdom	Cancer surgery and infusion COEs; Caledonia/Essex highest poverty; broadband and telehealth priority
Springfield	Springfield Hospital	\~31K, rural, Windsor/Windham border	Memory care and psych-adult COEs; high SUD corridor; transformation plan most complex given financial position
Morrisville	Copley Hospital	\~31K, rural, Lamoille/Orleans	Orthopedics and rheumatology COEs; Stowe/ski area seasonal population; workforce housing critical
Randolph	Gifford Medical Center	\~28K, very rural, Orange County	TBD tier — among most financially vulnerable; comprehensive assessment needed; AHEAD CAH protections important
Grace Cottage	Grace Cottage Hospital	\~14K, very rural, Windham County; 19-bed Vermont's smallest hospital	REH conversion most likely path; coordination with Brattleboro Memorial

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
			essential; palliative care specialty worth preserving

Figure 16.3 — Vermont 14-Hospital Service Areas: regional profile and priority transformation challenges for AHS HSA-level coordination. Sources: Oliver Wyman Act 167 Report; Vermont RHT Program Application; AHS Transformation Reports 2025\.

Level 3 — Program Delivery Operator

At the program level, AHS continues to administer Vermont's healthcare and social service programs — Medicaid (through DVHA), mental health (through DMH), Blueprint for Health, developmental services, aging services, and others. The structural change is not the elimination of program-level operations but the addition of cross-program coordination mechanisms and the explicit alignment of program operations with the transformation goals and geographic structures established at the statewide and regional levels.

The critical program-level change is the integration of social service programs into the healthcare transformation framework. Oliver Wyman's three imperatives — build housing and fix transportation, pay with RBP and move to global budgets, move care out of hospitals — cannot be achieved by the healthcare programs alone. DVHA, Blueprint, and the Health Care Reform Office can transform payment and care delivery; they cannot build housing or run transportation programs. But the AHS programs that fund housing supports (LIHEAP, general assistance), transportation (Medicaid transportation, EMS), food assistance (SNAP administration), and developmental services are sitting in the same agency — they are simply not coordinated with the healthcare transformation agenda.

The specific structural mechanism for this integration is the HSA Coordinator role: a designated AHS staff member for each HSA region who convenes the relevant program staff from DVHA, DMH, Blueprint, ADS, DAIL, and the Health Care Reform Office around the hospital transformation plan for their region. The HSA Coordinator does not control all program resources — that would require a reorganization far beyond what Act 68 mandates — but they create the coordination layer that currently does not exist.

The Six-Pillar Framework Applied to AHS's Restructuring Mandate

The six-pillar framework — Policy, Economics, Operations, Clinical, Equity, and Technology — provides the analytical architecture for AHS's Statewide Strategic Plan. Each pillar corresponds to a domain of AHS's transformation mandate, a set of organizational capabilities AHS must develop, and a set of measurable outcomes AHS must track. The following section maps each pillar explicitly onto AHS's restructuring requirements.

Pillar 1 Policy	AHS function: Legislative compliance, federal relationship management, regulatory strategy
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AHS's policy pillar function is to ensure that Vermont's transformation agenda is legally grounded, federally funded, and legislatively supported. The specific organizational requirements are:

- AHEAD Model management: Active management of Vermont's AHEAD State Agreement, including ongoing CMS negotiation on global budget methodology, monitoring compliance with TCOC targets, and managing the conditions under which Vermont can withdraw if the model is not working. This requires dedicated staff with both healthcare finance expertise and federal program management experience.
- * Act 68 compliance tracking: Systematic monitoring of Act 68's statutory requirements — RBP by FY2027, global budgets for non-CAH hospitals by FY2028, Statewide Strategic Plan by December 2028 — and proactive management of the legislative reporting calendar.
- * RHT Program administration: Oversight of Vermont's \$195M five-year award, including grant management, contractor oversight, annual reporting to CMS, and alignment of RHT activities with Act 167 and Act 68 goals. This is a major federal grant program requiring professional grant management infrastructure.

* Federal policy monitoring: Tracking changes to Medicare, Medicaid, and federal healthcare policy that affect Vermont's transformation — including Medicaid reconciliation legislation, CMMI model changes, and CMS regulatory guidance — and updating Vermont's strategy in response.

Pillar 2 Economics	AHS function: Payment reform sequencing, financial sustainability monitoring, ROI analysis
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AHS's economics pillar function is to manage Vermont's payment reform transition — coordinating with GMCB on RBP and global budget design, monitoring hospital financial sustainability, and ensuring that payment reform produces the premium reduction and affordability improvement that Act 68 requires.

- Payment reform coordination: Regular coordination with GMCB on RBP methodology development, global budget design, and the premium passthrough monitoring that Act 68 requires. AHS brings the Medicaid perspective and the population health data; GMCB brings the regulatory authority. Neither can execute the economics transformation without the other.

* Hospital financial monitoring: Tracking Vermont hospitals' financial positions against the Oliver Wyman projections — particularly the CAH operating profit gap (\$938 vs. \$2,784 benchmark) and the PPS administrative cost premium (\$2,730 vs. \$1,427 benchmark) — and intervening with technical assistance or grant support before hospitals reach crisis.

* AHEAD EAST Fund management: Directing the up to \$150M annually in additional Medicare funds toward the primary care, behavioral health, home health, and long-term care investments that make global budgets produce population health improvement rather than just cost reduction.

* Transformation ROI tracking: Measuring whether Vermont's transformation investments — RHT Program, Act 68 grants, AHEAD EAST Fund — are producing the \$400M+ in savings that Oliver Wyman projected. This requires the analytics capability described in the organizational gap analysis.

Pillar 3 Technology	AHS function: Data infrastructure, HIE governance, analytics platform, EHR strategy
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AHS's technology pillar function is to ensure Vermont has the data infrastructure needed to manage a transformed healthcare system. The current data environment — VHCURES as the all-payer claims database, VITL as the health information exchange, the Blueprint clinical data registry, and fragmented program-level data systems — was adequate for monitoring discrete programs. It is not adequate for managing population health at the system level.

- VHCURES expansion and real-time access: AHS and GMCB must expand VHCURES to include more timely claims data, enhance provider connectivity (currently voluntary and incomplete), and make VHCURES data accessible for transformation planning analytics. The November 2025 AHS report acknowledged that most available measures reflect 2023 or earlier data — a two-year lag that makes real-time transformation management impossible.

* HIE strategic development: The Act 167-mandated HIE feasibility assessment — currently evaluating the cost-benefit of a statewide electronic health record — is a technology pillar decision with major implications for the Strategic Plan. A statewide EHR would resolve the fragmented data environment that Oliver Wyman cited as a barrier to coordinated care; the question is whether the investment is justified and whether the governance model is viable.

* Analytics vendor implementation: The joint AHS-GMCB analytics vendor procurement underway must produce a platform capable of modeling transformation scenarios, evaluating hospital proposals, and generating the outcome metrics the Strategic Plan requires. This is the most urgent near-term technology investment.

* Cross-program data integration: AHS's SDOH programs (housing, transportation, food assistance) must be connected to healthcare outcome data so that the relationship between social service investment and healthcare utilization can be measured. Without this integration, AHS cannot demonstrate to the legislature that its SDOH investments are producing healthcare savings.

Pillar 4 Clinical	AHS function: Primary care expansion, behavioral health integration, care model design
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AHS's clinical pillar function is to design and support the care delivery models that Vermont's transformed payment system requires. Payment reform creates the incentive for clinical redesign; AHS must create the programmatic infrastructure that makes clinical redesign possible.

- Blueprint for Health expansion and AHEAD transition: Managing the transition of Blueprint's Medicare payments from the current demonstration to Primary Care AHEAD, expanding PCMH enrollment, and ensuring that the MHI pilot's behavioral health integration is made permanent through AHEAD EAST Fund investment.
- * CCBHC network development: Executing the plan to certify five additional CCBHCs by July 2026, covering all major HSA regions, and integrating the CCBHC network with primary care, hospital emergency departments, and the 988 crisis system.
- * Care model technical assistance: Supporting hospitals and primary care practices in redesigning their care models for the global budget environment — shifting from volume maximization to population health management, building care coordination capacity, and implementing the team-based care models that primary care transformation requires.
- * Long-term care capacity development: Addressing the PACE gap, supporting dementia care COE development, and working with DVHA to expand HCBS capacity so that Vermont's growing elderly population can age in their communities rather than consuming hospital capacity.

Pillar 5 Equity	AHS function: Disparity elimination, geographic access, SDOH programs, culturally competent care
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AHS's equity pillar function is to ensure that Vermont's transformation reduces rather than exacerbates health disparities — particularly the geographic disparities between Burlington's Chittenden County and the Northeast Kingdom, and the access disparities affecting Vermont's Medicaid population, rural residents, and linguistic minorities.

- Geographic equity monitoring: Tracking health outcomes, primary care access, and insurance coverage rates by county — with specific attention to the Northeast Kingdom (6-8% uninsurance vs. 3% statewide) and rural counties with higher rates of behavioral health crisis presentations, opioid overdose, and diabetes.
- * Medicaid access protection: Monitoring whether RBP and global budget implementation affects Medicaid patients' access to care — specifically whether the reduction in commercial prices affects hospitals' willingness to maintain Medicaid patient volumes. Act 68 explicitly requires consideration of access protection in the RBP design.
- * Culturally competent care investment: Addressing the equity gaps Oliver Wyman documented — lack of language access, cultural mismatch in clinical care, inadequate LGBTQ+ competency, weight bias — through workforce investment (provider diversity recruitment), training requirements, and community partnership programs.
- * SDOH program coordination: Ensuring that AHS's SDOH programs — housing supports, transportation, food assistance — are explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization and health disparities.

Pillar 6 Operations	AHS function: Transformation plan management, RHRC successor, administrative simplification
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AHS's operations pillar function is to manage the execution of Vermont's transformation — the 14 hospital transformation plans, the regional coordination processes, the technical assistance programs, and the administrative simplification agenda that Oliver Wyman identified as essential for reducing provider burden.

- Hospital transformation plan oversight: Managing the pipeline from Draft Care Transformation Strategy Outlines through Draft Plans to Final Plans for all 14 hospitals, ensuring plans are coordinated regionally and nationally, and tracking implementation against Act 167 outcome objectives.
- * Administrative simplification: Acting on Oliver Wyman's recommendation to further simplify prior authorization, the CON process, and Medicaid billing — reducing the administrative burden that drives physician frustration and consumes provider resources that should go to patient care.

* Project Management Office: Establishing the PMO infrastructure needed to manage Vermont's transformation portfolio — RHT Program, AHEAD implementation, hospital transformation plans, Strategic Plan development — with professional project management, risk tracking, and escalation processes.

* EMS and transportation transformation: Coordinating the EMS professionalization and regionalization that Oliver Wyman prioritized — addressing the 31-agency fragmentation that makes inter-facility transfers complex and expensive, and building the transportation infrastructure that population health management requires.

The AHS Restructuring Implementation Timeline

The AHS restructuring is not a future event — it is happening now, driven by statutory deadlines that are already creating organizational pressure. The following timeline organizes the key milestones that AHS must meet, grouped by the organizational capability they require.

Deadline	Milestone	Primary owner	Status
FY2026 (now)	Hospital transformation grant awards; all 14 hospitals engaged in transformation planning	AHS Health Care Reform Office	In progress — all eligible hospitals submitted applications
FY2026 (now)	Analytics vendor procurement (joint AHS-GMCB)	AHS \+ GMCB	Procurement underway as of November 2025
FY2026 (now)	CCBHC certification for 5 new entities (July 2026 target)	AHS Department of Mental Health	On track per RHT application plan
FY2026 (now)	AHEAD Medicaid global budget operational (January 2026\)	DVHA / CMS	Operational — Medicaid global budget in effect
FY2026 (now)	RHT Program implementation planning and staff hiring	AHS Health Care Reform Office	\$195M award received December 2025; implementation planning underway
FY2026 Q2-Q4	Final hospital transformation plans (early 2026\)	AHS \+ 14 hospitals \+ RHRC successor	Plans expected early 2026; AHS integration work follows
FY2026 Q2-Q4	HSA Coordinator roles established	AHS Secretary	Organizational change requiring Secretary-level decision
FY2026 Q2-Q4	Division of Planning and Effectiveness established	AHS Secretary \+ GMCB Chair	Recommended by OW; critical for Strategic Plan development
FY2027 (Oct 2026\)	RBP maximum rates in effect for commercial payers	GMCB (AHS coordination)	Statutory deadline; GMCB leads; AHS coordinates on Medicaid alignment
FY2027 (2027)	AHEAD Model Cohort 2 begins — Medicare FFS global budgets	AHS \+ DVHA \+ GMCB \+ CMS	January 1, 2027; nine-year model; requires ongoing federal coordination

Deadline	Milestone	Primary owner	Status
FY2027 (2027)	Primary Care AHEAD operational — enhanced primary care payments begin	DVHA \+ Blueprint for Health	Medicare primary care payments through AHEAD replaces expiring demo
FY2028 (Oct 2027\)	Global hospital budgets for non-CAH hospitals	GMCB (AHS coordination)	Statutory deadline; requires completed RBP methodology and budget design
December 2028	Statewide Health Care Delivery Strategic Plan delivered to legislature	AHS Secretary \+ Health Care Delivery Advisory Committee	Statutory deadline; updated every three years thereafter
FY2030 (Oct 2029\)	Global hospital budgets for all Vermont hospitals including CAHs	GMCB (AHS coordination)	Statutory deadline; completes all-hospital global budget coverage

Figure 16.4 — AHS restructuring and Vermont transformation implementation timeline. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Reports; Vermont RHT Program Application.

A Framework for the 2028 Statewide Health Care Delivery Strategic Plan

The Statewide Health Care Delivery Strategic Plan is the most consequential document AHS will produce in the 2020s. It will define Vermont's healthcare system for the decade following its delivery. The following framework provides the structural architecture for the plan, organized around the six pillars and Act 68's statutory requirements.

Proposed Plan Structure

Section	Content	Length/depth
Executive Summary	Vermont's healthcare crisis in 2028: what changed since 2022; what the system looks like today; the five Act 167 goals and progress toward each; the plan's central commitments for 2028-2031	5-8 pages; accessible to general public and legislators
Section 1: System Context and Diagnosis	Updated version of the Oliver Wyman analysis: Vermont's demographic trajectory, hospital financial positions, payer landscape, workforce status, and SDOH burden as of 2028 baseline. What has improved; what remains urgent.	15-20 pages; includes data tables and visualizations
Section 2: Payment Reform Architecture (Economics Pillar)	Status of RBP implementation; global budget parameters for each hospital; AHEAD TCOC targets and performance to date; total cost of care trajectory 2028-2031;	20-25 pages; includes hospital-by-hospital financial data

Section	Content	Length/depth
	affordability benchmarks and progress	
Section 3: Primary Care and Behavioral Health (Clinical Pillar)	Blueprint enrollment and performance; CCBHC network coverage; CoCM deployment status; Primary Care AHEAD performance; behavioral health access indicators and improvement targets 2028-2031	15-20 pages
Section 4: Hospital Network and Regionalization (Operations Pillar)	Confirmed tier designations for all 14 hospitals; COE network status; transfer infrastructure; shared services implementation; REH/CACC conversions completed or planned; workforce distribution	20-25 pages; includes hospital network map
Section 5: Data and Technology Infrastructure (Technology Pillar)	VHCURES enhancement status; HIE connectivity; statewide EHR decision; analytics platform capabilities; population health data architecture; interoperability with federal systems	10-15 pages
Section 6: Health Equity Strategy (Equity Pillar)	Geographic equity indicators by HSA; disparity measures by income and demographics; SDOH program coordination framework; culturally competent care investments; equity targets 2028-2031	15-20 pages
Section 7: Workforce Strategy	Current workforce supply and demand by profession and geography; RHT Program workforce investments and outcomes; scope-of-practice changes; pipeline programs; 2030 workforce projections	15-20 pages
Section 8: AHS Organizational Architecture (Policy and Operations Pillars)	AHS organizational structure as redesigned; HSA Coordinator model; Division of Planning and Effectiveness; cross-program SDOH integration; governance relationships with GMCB, DVHA, DMH, DFR	10-15 pages
Section 9: Accountability Framework	Complete metrics dashboard (all 17+ Act 167/Act 68 outcome measures); data sources and update frequencies; performance targets for 2028-2031;	10-15 pages; appendix with full metrics table

Section	Content	Length/depth
	reporting calendar; conditions for plan revision	
Section 10: Financial Plan	Capital allocation plan for RHT Program (remaining 3-year balance); AHEAD EAST Fund allocation; Act 68 grant commitments; projected system savings from transformation; state budget implications	15-20 pages

Figure 16.5 — Proposed structure for Vermont's 2028 Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate; six-pillar framework.

The Central Commitment the Plan Must Make

Beyond its analytical content, the Strategic Plan must make one central commitment that is currently absent from Vermont's transformation discourse: a specific, quantified trajectory for what Vermont's healthcare system will look like in 2031, 2035, and 2040, with the financial modeling to show it is achievable.

Oliver Wyman provided the diagnostic scenario: without transformation, 13 of 14 hospitals report losses by 2028, premiums reach unsustainable levels, and the healthcare system contracts through unplanned closures rather than orderly redesign. The Strategic Plan must provide the alternative scenario with equal specificity: if Vermont executes the transformation agenda on the timelines described in this chapter, what does the system look like in 2031? How many hospitals maintain inpatient beds? What is the primary care investment level? What is the premium growth rate? What is the uninsurance rate in the Northeast Kingdom?

This is the difference between a strategic plan and a strategic aspiration. Vermont's legislators, hospital leaders, community organizations, and Vermonters deserve to know not just what AHS is doing but what the state will look like if AHS succeeds. The six-pillar framework provides the analytical architecture for making that commitment with intellectual honesty about what is achievable and what remains uncertain.

Statutory deadline: Statewide Strategic Plan delivered to legislature Dec 2028 Act 68 of 2025 — updated every three years thereafter	Health Care Delivery Advisory Committee: members 18 AHS, GMCB, HCA, hospitals, health centers, insurers, professions, small business, LTC
RHT Program capital available over 5 years for transformation \$195M Vermont's December 2025 award; aligned with Act 167 and Act 68 goals	Oliver Wyman projected direct savings from transformation (5 years) >\$400M Independent of AHEAD and RBP payment reform savings

Figure 16.6 — Key parameters for AHS's Strategic Plan development. Sources: Act 68; VTDigger; Oliver Wyman Act 167 Report; Vermont RHT Program Application.

Vermont as National Template — Why This Restructuring Matters Beyond Vermont

Vermont's AHS restructuring is not a local administrative exercise. It is the first attempt by a state agency to systematically redesign itself as a population health system operator in response to a statutory mandate for healthcare transformation. Every state that faces Vermont's demographic and fiscal trajectory — which is to say, every state in the Northeast, most of the Midwest, and eventually every state — will need to answer the same organizational design questions Vermont is answering now.

The questions are not Vermont-specific. How do you align a state human services agency around geographic service areas rather than program silos? How do you build the analytics capacity to model the impact of structural change before you implement it? How do you integrate social service programs with healthcare delivery reform so that SDOH investments actually reduce healthcare utilization? How do you manage a multi-stakeholder strategic planning process that includes

hospitals, insurers, providers, and community organizations without producing a lowest-common-denominator plan that satisfies everyone and commits to nothing?

Vermont will have answers to these questions by December 2028 — answers grounded in real experience with a real system facing real pressures. Those answers will be the most valuable policy contribution Vermont has made to American healthcare in a generation. The national significance of Vermont's transformation is not that Vermont is large or economically powerful. It is that Vermont is early — early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to still be actionable.

This is why the book you are reading exists: to document Vermont's experience in a form that is analytically rigorous, practically useful, and transferable to the healthcare leaders and policymakers who will face Vermont's challenges in their own states and systems. The AHS restructuring chapter is not the end of the story — it is the moment when the story becomes fully operational.

Key Concepts Introduced in This Chapter

Term	Definition
Population health system operator	The organizational identity AHS must adopt — managing Vermont's healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically, and accountable for measurable progress toward the five Act 167 goals.
HSA Coordinator	A proposed AHS organizational role: a designated staff member for each Hospital Service Area who convenes the relevant AHS programs and community partners around the hospital's transformation plan, creating the cross-program coordination layer that currently does not exist.
Division of Planning and Effectiveness	A proposed new organizational unit within AHS (also recommended as an addition to GMCB in Oliver Wyman's report) to provide the analytical capacity for modeling transformation scenarios, evaluating hospital proposals, and monitoring outcome metrics.
Statewide Health Care Delivery Strategic Plan	The plan AHS is required by Act 68 to deliver to the Vermont legislature by December 1, 2028, and update every three years. The plan must address Vermont's healthcare delivery system across all dimensions: payment reform, primary care, hospital network, behavioral health, equity, workforce, and AHS organizational architecture.
Health Care Delivery Advisory Committee	The 18-member committee established by Act 68 to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Has statutory authority to set affordability benchmarks and advise the legislature directly on healthcare delivery.
Program-silo structure	AHS's current organizational model, organized around funding streams and program types rather than geographic service areas or population segments. Oliver Wyman identified this as a primary barrier to coordinated transformation.

Term	Definition
Cross-program SDOH integration	The organizational mechanism for connecting AHS's social service programs — housing, transportation, food assistance — with healthcare transformation goals, so that SDOH investments can be explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization.
Reform cascade	The legislative sequence — Act 167 (2022) → Act 51 (2023) → Act 68 (2025) — in which each intervention builds on the evidence and institutional capacity created by its predecessor. Vermont's AHS restructuring is the organizational expression of this legislative cascade.

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services Health Care Delivery Advisory Committee page (healthcarereform.vermont.gov); AHS Health Care System Transformation Reports (August 2025, November 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); AHEAD State Agreement (January 17, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); GMCB Act 68 Update on RBP and Global Budgets (February 17, 2026); All prior chapters of this book.

Appendix A: Vermont System Portrait

Demographics, Hospitals, Insurance, Workforce, and System in Motion

How to use this appendix

This appendix consolidates the Vermont-specific factual foundation for the book's national argument. Chapter 2 embeds only the fiscal crisis data needed to understand why sequencing matters in Vermont; this appendix provides the complete system portrait that Chapter 2 and subsequent chapters draw on. References throughout the book to Vermont's demographics, hospital system, regulatory architecture, and workforce data point here. Each section is self-contained and can be read independently.

Former Chapter 2 content (Vermont System Portrait) has been fully integrated here, supplemented by the January 2026 hospital transformation plan data from AHS tracking records and the GMCB regulatory architecture from ch2_additions.

A.1 Vermont: The State

Vermont is the second-smallest state by population in the United States. Its 647,000 residents live across 14 counties and 255 towns and cities in a predominantly rural landscape — 9,616 square miles of mountains, valleys, farms, and forest. The population density is 70 people per square mile, well below the national average of 94. Nearly two-thirds of Vermont residents live in areas classified as rural by the federal definition used for healthcare program eligibility.

Vermont's geography is not merely a demographic fact. It is a healthcare infrastructure constraint. A patient in Essex County — one of the most rural and least populated counties in the Northeast Kingdom, with an uninsurance rate of 8% compared to Vermont's statewide 3% — may be 45 minutes from the nearest hospital emergency department, 90 minutes from a specialist, and hours from a tertiary care center. Public transportation is limited or unreliable in most rural counties. Broadband penetration reaches 80% of rural areas, leaving 20% without the connectivity that telehealth and remote patient monitoring require.

Vermont's economy is diverse but small. Healthcare is one of the most common industries in the state — employing approximately 14% of the workforce and representing a major economic anchor in rural communities where the hospital is often the largest employer. The University of Vermont Medical Center in Burlington, with approximately 50% of the state's hospital market share, is the economic center of gravity of Vermont's healthcare system. Every reform that affects UVMHC affects the Vermont economy in ways that hospitals in larger, more economically diverse states do not.

A.2 Vermont Demographics and Population Projections

Vermont's demographic trajectory is the foundational fact of its healthcare transformation challenge. Oliver Wyman's population projections, derived from MPR Vermont population data, tell the story with precision.

Indicator	Value	Significance
Vermont population aged 65+ by 2040	30%+	Up from 21.7% in 2020 — a 57% increase in the elderly population
Working-age population change by 2040	−13%	From 367,000 to approximately 318,000 — the commercial insurance base shrinks
Total Vermont population 2020–2040	Flat	Approximately 612,000–624,000 throughout — aging, not growing
Vermont residents with health insurance (2025)	97%	One of the highest coverage rates in the country — ACA and Medicaid expansion impact

Primary care provider shortage projected by 2030	370 FTE	112 in family medicine, 190 in other primary care specialties (RHT Program application)
Rental vacancy rate	3%	Below healthy 5% threshold — housing as healthcare workforce constraint

Figure A.1 — Vermont demographic transformation: key projections and their healthcare implications. *Source: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.*

The implications of these demographic trends are not complicated — they are simply severe. An aging population means a shift in the payer mix toward Medicare and Medicaid, both of which reimburse hospitals below cost. A shrinking working-age population means a shrinking commercially insured population, reducing the cross-subsidy that currently sustains hospital operations. More complex care needs among the elderly means higher costs per patient. And an aging healthcare workforce — Vermont has the same age distribution among its clinicians as among its population generally — means accelerating retirements of experienced providers in exactly the years when demand is rising.

Oliver Wyman's analysis named this dynamic directly: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift. The commercial premiums that make it possible for hospitals to accept below-cost Medicare and Medicaid payments depend on a sufficient pool of commercially insured patients. As that pool shrinks relative to the Medicare and Medicaid population, the math stops working — not in a distant future scenario, but on the trajectory Vermont is already on.

A.3 Vermont's Regulatory Architecture: The Green Mountain Care Board

Vermont's healthcare transformation would be impossible without an institutional actor that most other states do not have. The Green Mountain Care Board — a five-member independent state body created in 2011 — is the regulatory engine that converts Vermont's legislative mandates into binding operational obligations for its hospital system. Understanding the GMCB is prerequisite to understanding why Vermont can attempt what other states cannot.

Vermont's hospital budgets have been subject to state review since 1983, but GMCB's assumption of regulatory authority beginning in fiscal year 2013 qualitatively changed the nature of that oversight. The GMCB does not merely review hospital budgets — it establishes them. Under Vermont statute, each hospital must operate within the budget the Board sets. When hospitals exceed those budgets, the GMCB can enforce by reducing future commercial rate approvals. When hospitals face financial distress, the GMCB Chair meets directly with hospital leadership. When the Legislature mandates transformation planning under Acts 167 and 68, GMCB is the institutional receptor for the resulting analysis and the body responsible for translating policy architecture into hospital-level operational requirements.

The practical significance of this regulatory architecture extends beyond Vermont's 14 hospitals. GMCB's annual budget review process, conducted entirely in public meetings under Vermont's Open Meeting Law with mandatory participation from the Office of the Health Care Advocate, has produced a documentary record of Vermont's hospital system that is unmatched in American health policy. For FY25, GMCB approved \$3.7 billion in system-wide hospital spending — a 4.1% increase after hospitals collectively requested 8%. The Board also enforced FY23 budget overages against UVMMC (\$80.3M) and RRMC (\$11.1M), reducing future commercial rate approvals as a consequence.

GMCB function	What it does and why it matters for transformation
Hospital budget review	Annually establishes Net Patient Revenue (NPR) growth limits for all 14 hospitals. For FY25, GMCB approved \$3.7 billion in hospital spending — a 4.1% increase after hospitals requested 8%. This regulatory compression drives hospital attention to efficiency and alternative revenue models.
Budget enforcement	When hospitals exceed approved budgets, GMCB can enforce by reducing future commercial rate approvals. In FY23, UVMMC exceeded its budget by \$80.3M and RRMC by \$11.1M — GMCB

	enforced both. This enforcement authority distinguishes Vermont's regulatory model from purely advisory state oversight.
Monthly performance monitoring	All 14 hospitals submit monthly financial performance reports throughout the year. This continuous monitoring capability is the operational substrate for Vermont's transformation accountability.
Reference-based pricing (RBP)	Act 68 mandates RBP beginning FY2027, with GMCB setting the methodology. RBP will cap commercial hospital rates as a percentage of Medicare. GMCB's existing rate review infrastructure makes mandatory RBP implementable.
Global budget design	GMCB will set prospective global hospital budgets beginning FY2028–2030, in coordination with the AHEAD Model. The analytics vendor procurement (McKinsey, 2025–2026) is GMCB building the technical infrastructure for global budget methodology.
Hospital sustainability planning	Under Act 167, GMCB commissioned the Oliver Wyman analysis and is the institutional receptor for its recommendations. GMCB's public meeting record is the documentary foundation for Vermont's transformation case.

Figure A.2 — GMCB regulatory functions and their significance for Vermont's transformation agenda. *Source: GMCB Hospital Budget Review Guide; Acts 167 and 68; GMCB Annual Report 2024.*

GMCB's institutional design explains a specific feature of Vermont's transformation that is analytically important: mandatory authority precedes payment reform deployment. The voluntary ACO era produced modest results because the highest-cost providers could decline to participate. GMCB's mandatory budget authority — its ability to set rates rather than merely recommend them — is the structural condition that makes Vermont's mandatory reference-based pricing and global budgeting achievable. Other states attempting payment reform without a comparable regulatory body are, in the framework's terms, attempting Economics pillar transformation without the Policy pillar architecture that gives payment reform enforcement teeth.

A.4 Vermont's Hospital System

Vermont has 14 general acute care hospitals, all of which are non-profit. This fact — no for-profit hospitals — is significant for the reform agenda: Vermont's hospital system does not have the shareholder value obligations that complicate transformation discussions in states with investor-owned hospital chains. Vermont's hospitals are community assets, governed by community boards, with legal missions oriented toward community health. This creates better conditions for the kind of collaborative transformation Vermont is attempting — but it does not eliminate the financial pressures, institutional interests, or organizational inertia that make transformation difficult everywhere.

Of Vermont's 14 hospitals, the University of Vermont Health Network comprises UVMMC, CVMC, and Porter Hospital as system hospitals. Dartmouth Health affiliates include SVMC and Mt. Ascutney. The remaining hospitals — Gifford, Copley, NCH, NVRH, BMH, Springfield, RPMC, Grace Cottage, and NMC — operate as independent community institutions or loose collaborative members, and it is these independent hospitals that face the most acute financial fragility.

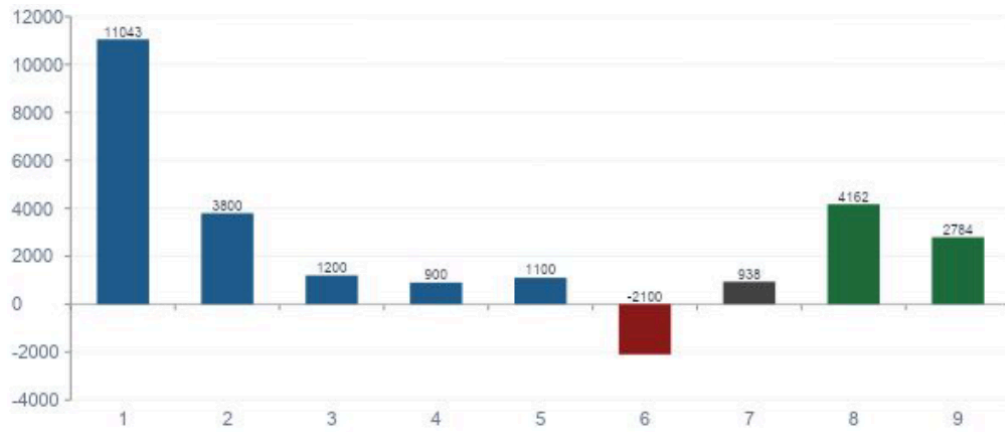


Figure A.3 — Vermont Hospital Financial Positions: FY2023 vs. national benchmarks. Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 10. AHS Health Care System Transformation Report (November 2025); NASHP via CMS.

The distribution of financial distress across hospital categories matters for reform design. Critical Access Hospitals — the smallest, most rural hospitals serving the communities with the fewest alternatives — are the most financially distressed, with an average operating profit of \$938 per adjusted discharge versus a \$2,784 benchmark. They are also the hospitals where service reductions or closures would have the most severe community consequences, and the hospitals that the AHEAD Model's global budget design must protect most carefully. UVMHC's administrative cost per adjusted discharge was \$3,826 against a national benchmark of \$1,427 — structural cost problems that voluntary reform cannot address.

The Fourteen Hospitals: Transformation Status and Challenges (January 2026)

The January 2026 transformation plans submitted to AHS under Act 68 provide a uniquely granular view of where each hospital stands. These are operational documents produced in response to a statutory mandate, reviewed by AHS staff, and directly tied to Rural Health Transformation Program grant funding. The following table characterizes all 14 hospitals by type, transformation plan grant status, primary focus, and key challenges as documented in the January 2026 submissions.

Hospital	Type	Grant	Primary transformation focus	Key challenges / concerns
UVM Medical Center (UVMHC)	Academic hub	Not eligible	Technology solutions for acuity management and transfers; statewide reference hub	Administrative cost \$3,826/adj. discharge vs. \$1,427 benchmark; FY23 budget overage enforcement (\$80.3M)
Central Vermont Medical Center (CVMC)	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers	Part of UVMHCN system; subject to system-level budget constraints
Porter Hospital	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers (shared strategy with CVMC)	Part of UVMHCN system; FY23 budget overage of \$11M
Rutland Regional Medical Center (RRMC)	Regional PPS	Awarded	Hospice utilization improvement; dermatology in community; regional collaboration with SVMC	FY23 budget overage of \$11.1M (partially enforced); exploring regional service lines with SVMC

SW Vermont Medical Center (SVMC)	Regional PPS	Awarded	RN Bridge to PCP program; \$29M cancer center; DaVinci robot; Epic EHR migration (\$30M); adolescent mental health unit	Locum provider costs; care coordination gaps; transport barriers for non-acute patients
Brattleboro Memorial Hospital (BMH)	Rural PPS	Awarded	Mobile Integrated Health (MIH) program; FQHC Look-Alike feasibility; birthing and oncology sustainability	GMCB FY26 deep concern about solvency; operating losses FY22/FY24/projected FY25; executive bonus scrutiny
Springfield Hospital	Rural PPS	Needs follow-up	Clinical services redesign feasibility study; relationship with Northstar Health	Oncology and cardiology sustainability concerns; plan not approved as of Jan 2026
Gifford Medical Center	CAH + FQHC	Awarded	Hospital-to-hospital transfer program with UVMHC/DHMC; NECHN GPO (\$793K projected savings); shared radiology	Targeting avg. daily census of 20 for sustainability; tele-radiology vendor capacity constraints
Copley Hospital	CAH	Awarded	Consulting services; primary care access expansion	Plan quality concerns; GMCB required FY25 budget modifications; sustainability of changes under discussion
North Country Hospital (NCH)	CAH	Awarded	Regionalization planning with NVRH; shared project manager and legal support; pulmonology; tele-neonatology	Limited IT staff; dependent on Vermont CIN shared services model for analytics capability
Northeastern VT Regional Hospital (NVRH)	CAH	Awarded	Regionalization planning with NCH; cardiology; tele-ICU	FY23 budget overage (not enforced); GMCB monitoring; limited workforce pipeline in Northeast Kingdom
Mt. Ascutney Hospital (MAHHC)	CAH + FQHC	Awarded	Age-Friendly Program / geriatric COE; Collaborative Care Model via DHMC; shared CEO/CFO/COO with Valley Regional (\$600K savings)	EMR transition (Oct 2025) disrupted data; subsidizing pharmacy (\$50K/yr); residential care facility support (\$950K in FY25)
Grace Cottage Hospital	CAH	Awarded	\$24M primary care clinic capital campaign; same-day access clinic; swing bed	Denial reduction \$76K→\$1,274 (FY25 success); FY24 loss of \$4M reduced to ~\$1M

			program; AI ambient scribe (Coverys grant)	in FY25; no DSH payments
Northwestern Medical Center (NMC)	CAH	Awarded	Innovative birthing services retention; shared cardiologist with Copley; oncology	Questions on value of marketing campaigns; schedule not yet confirmed as of Jan 2026

Figure A.4 — Vermont's 14 hospitals: typology, transformation grant status, primary focus, and key challenges. *Source: AHS Hospital Tracking data (January 2026); individual hospital transformation plans (January 15, 2026); GMCB FY26 Budget Orders.*

TIER 1	Regional Specialty Centers (RSC) UVMHC (16 COEs) SW Vermont Medical Brattleboro Memorial Rutland Regional Central VT Medical Highest COE count. Accept referrals from the network for designated specialties. Financial sustainability strongest. Primary RSC role: UVMHC.
TIER 2	Community Hospitals — Focused Scope NW Medical Center NE VT Regional Springfield Copley Mt. Ascutney Porter Medical 2–4 COE designations. Maintain specific service lines. Transfer complex cases to RSCs. Actively engaged in Act 68 transformation planning.
TIER 3	Transition Assessment Required Grace Cottage (19 beds) Gifford Medical North Country Hospital Most financially vulnerable. REH conversion, CACC, or home-based care model under assessment. Act 68 transformation plans address transition path.

Figure A.5 — Vermont's 14-Hospital Network: Three-Tier Structure (Tier 1 Regional Specialty Centers, Tier 2 Community Hospitals, Tier 3 Transition Assessment). *Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 11. Oliver Wyman Act 167 Report (2024); AHS November 2025 Transformation Report.*

Patterns in the Data

The administrative cost problem is concentrated at the top of the system. Oliver Wyman's finding that UVMHC's administrative cost per adjusted discharge was \$3,826 against a national benchmark of \$1,427 is not merely an efficiency observation. It is evidence that Vermont's largest and most financially powerful hospital has structural cost problems that voluntary reform cannot address. GMCB's enforcement of UVMHC's FY23 budget overage, and its decision to not approve NPR growth for BMH in FY26 while expressing deep concern about BMH's solvency, illustrate the regulatory tools available when transformation accountability is backed by statutory authority.

The regionalization logic is visible in the transformation plans but is not yet architecturally formed. NCH and NVRH are explicitly partnering to share a project manager, legal counsel, and subject matter expertise for regionalization planning. Gifford has built a functioning hospital-to-hospital transfer protocol with UVMHC and Dartmouth Health. Mt. Ascutney and Valley Regional Hospital have shared their CEO, CFO, and COO since late 2024, generating approximately \$600,000 annually in administrative savings. SVMC and RRMC are exploring shared specialty and support service lines. These are the early formations of the regional hub structure that Acts 167 and 68 envision, but they are bottom-up and incomplete rather than top-down and comprehensive.

The analytics gap is an operational reality, not a theoretical concern. The AHS Hospital Transformation Analytical Support RFP, issued in late 2025 and awarded to McKinsey, acknowledged that Vermont does not yet have the analytics infrastructure to manage global budgets, measure population health outcomes at the hospital level, or track patient migration across its 14-institution system. The scope of that procurement — VHCURES integration, facility risk profiling, patient migration modeling, scenario modeling — reveals both the ambition of Vermont's transformation and the distance between current analytics capability and what AHEAD's launch in January 2027 will require.

A.5 The Transformation Plans: Vermont's System in Motion (January 2026)

The 148 individual transformation initiatives documented across Vermont's hospital plans in January 2026 are not policy proposals. They are operational commitments with named responsible parties, defined timelines, cost estimates, and measurement frameworks — produced under statutory obligation and reviewed by AHS staff who rate their quality and track their progress. This is the Operations pillar working in real time.

Initiative theme	Initiative count	Representative examples from hospital plans
Clinical service redesign	~55	SVMC RN Bridge to PCP; BMH MIH formalization; MAHHC Age-Friendly Program; Grace Cottage same-day access clinic; RPMC hospice utilization improvement
Regionalization / transfers	~20	NCH-NVRH shared regionalization planning; Gifford hospital-to-hospital transfers with UVMC/DHMC; MAHHC shared senior leadership with Valley Regional; SVMC-RPMC regional service lines (allergy, ENT, pain, ERCP)
Workforce development	~25	MAHHC Nursing Pathways Program (\$190K for two cohorts); SVMC 'grow our own' RN pipeline; Grace Cottage near-elimination of travel nurses; Mt. Ascutney SANE forensic capacity building
Technology and data	~20	SVMC Epic migration (\$30M); CVMC/Porter acuity management technology; Grace Cottage EMR interoperability interfaces; Grace Cottage ambient AI scribe (Coverys grant)
Shared purchasing / operational efficiency	~15	NECHN GPO launch (Gifford projected \$793K savings); captive insurance pool across NE Collaborative members; MAHHC-Valley Regional shared C-suite (\$600K annual savings)
Value-based care / payment alignment	~13	SVMC Belong Health Medicare Shared Savings ACO (2026); SVMC Medicaid global budget participation; Grace Cottage ACO participation and VATICA integration; Gifford FQHC model expansion

Figure A.6 — Vermont hospital transformation initiative themes, January 2026. Based on 148 initiatives from AHS Transformation Plans Analysis. Source: *AHS Transformation_Plans_Analysis.xlsx* (January 2026).

Three case studies in transformation from primary sources

Grace Cottage Hospital reduced insurance denials from \$76,160 in FY2024 to \$1,274 in FY2025 — a 98% reduction — through concurrent review for acute admissions, documentation improvement, and provider education. Its financial trajectory — from a \$4 million pre-audit operating loss in FY24 to approximately \$1 million in FY25 to projected break-even in FY26 — with no DSH payments and a 5-star National Rural Health Star Rating, makes it the book's most important counterexample to the narrative that Vermont's small CAHs are uniformly failing.

Mt. Ascutney Hospital's Community Health Team data — 1,132 individuals served over ten months in 2025, 80% over age 64, 22% requiring six or more contacts — is the most concrete available illustration of Vermont's demographic aging problem at the clinical level. The CHT is not serving a theoretical future population; it is already serving a population whose overwhelming predominance is geriatric, complex, and high-utilizing.

Mt. Ascutney's shared senior leadership arrangement with Valley Regional Hospital — shared CEO since December 2024, shared CFO since November 2025, shared COO since January 2026 — generating \$600,000 annually in administrative savings, while simultaneously subsidizing Windsor's only pharmacy (\$50K/yr) after the town's private pharmacy closed in 2025. This is the equity problem that rural hospital transformation must address: a small CAH bearing community infrastructure costs that its operating revenue cannot sustainably support.

What the Plans Reveal About the System's Constraints

Interoperability remains a solved problem only on paper. Grace Cottage's plan documents the reality with precision: working with multiple regional partners who operate on different electronic medical records, "the process from order to completion is arduous and full of administrative waste" — relying on "antiquated and insecure technologies" because secure interoperability infrastructure does not yet connect Vermont's system. Vermont's HIE, VTIL, exists but is not functionally integrated with the workflows of most community hospitals. This is not a technology problem awaiting a solution — the solutions exist. It is an incentive and governance problem: no single hospital can justify the cost of building interoperability infrastructure alone, but the collective benefit to the system is substantial. This is a textbook Technology-Operations dependency failure.

Community pharmacy access has emerged as a new access gap. Mt. Ascutney is subsidizing Windsor's only pharmacy after the town's private pharmacy closed in 2025. Grace Cottage's pharmacy fills over 6,000 scripts monthly as a community service, collecting patients from Brattleboro who have lost local pharmacy access. When a hospital becomes a community's pharmacy, its cost structure reflects that community function in ways that standard efficiency benchmarks do not capture.

The evidence that voluntary participation cannot achieve structural change runs through the plans like a thread. BMH's MIH program explicitly identifies sustainable funding as its primary barrier, noting that standard billing cannot cover the cost and requesting that AHS provide reimbursement through RHTP funding. NVRH and NCH's regionalization work explicitly identifies legal counsel and project management as shared costs they cannot individually afford. These are not capability failures — they are financing failures. The hospitals know what needs to be done. The current financial architecture makes it impossible to fund. This is precisely the condition that Acts 167 and 68 are designed to address.

A.6 The Insurance and Premium Crisis

Vermont's health insurance market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. Vermont was an early adopter of the ACA, expanding Medicaid in 2014 and establishing Vermont Health Connect, the state's insurance exchange. The result is a 97% coverage rate — one of the highest in the country — that is a genuine policy achievement.

The achievement of near-universal coverage has not, however, resolved Vermont's affordability crisis. Coverage is not the same as affordability. Vermont's individual market monthly premium rose from \$456 in 2018 to \$948 in 2024 — a 108% increase — while median household income grew 22%. Out-of-pocket maximums more than doubled. The insured Vermonter who delays necessary care because of the cost of their deductible or co-pay is not a healthcare access success story; they are an indictment of a system that has achieved coverage on paper while allowing costs to rise beyond reach.

The mechanism connecting hospital prices to insurance premiums is not complicated. Vermont hospitals charge commercial insurers 250–300% of Medicare rates on average for hospital services. In the Burlington area, UVMMC charged approximately 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. When a hospital charges 417% of Medicare for outpatient imaging, every insurer that covers that hospital's patients must charge premiums high enough to pay those prices and still maintain solvency. The result

is the premium trajectory Vermont has experienced: driven primarily by hospital price increases that have no regulatory brake. This is what reference-based pricing is designed to address, and why Act 68's mandate to implement RBP represents the most consequential health policy legislation Vermont has passed since Act 48 created the GMCB in 2011.

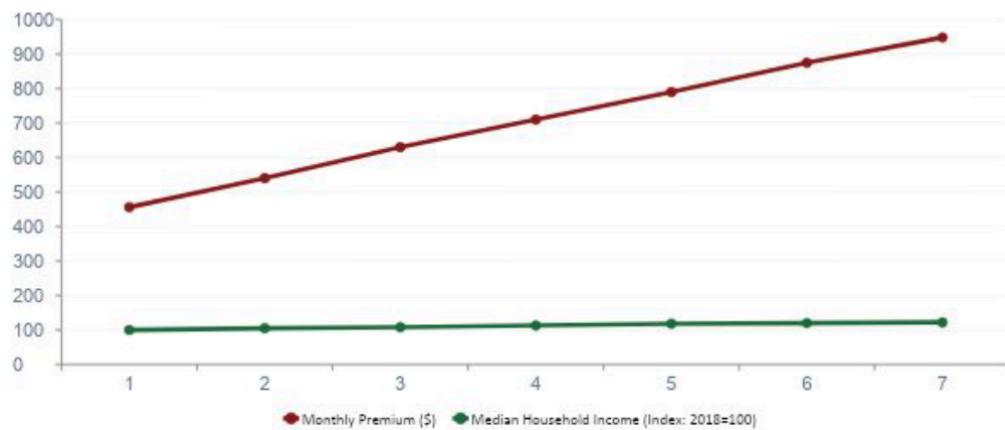


Figure A.7 — Vermont Premium vs. Income Growth 2018–2024: 108% premium increase vs. 22% income growth — the affordability gap driving Vermont's transformation mandate. Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 12. AHS January 2025 Legislative Presentation; GMCB rate filings.

A.7 Vermont Healthcare Workforce

Vermont's healthcare workforce faces a triple constraint that no single intervention can resolve: insufficient supply of primary care physicians, a specialist shortage that cannot be addressed by adding more specialists to each community, and a housing and economic environment that makes recruiting and retaining clinical staff increasingly difficult.

The primary care shortage is the most immediately actionable. Vermont's RHT Program application projects a shortage of 370 full-time equivalent primary care providers by 2030 — 112 in family medicine and 190 in other primary care specialties. Oliver Wyman's analysis offered a counterintuitive but important qualification: HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is not purely one of physician headcount — it is partly one of primary care productivity and care model. If primary care providers were supported to see three patients per hour in team-based care settings where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate primary care supply for well into the future.

The specialist shortage requires a different solution: regionalization. Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at every one of its 14 hospitals. The patient volume at smaller community hospitals does not justify the positions, and the clinical volume needed to maintain specialist expertise cannot be generated at a hospital serving 30,000–50,000 people. Oliver Wyman's Center of Excellence framework — concentrating specialty services at facilities with sufficient volume — is the correct response. The key is ensuring that concentration does not mean inaccessibility: telehealth consultation, reliable EMS transport, and clear referral pathways are the operational complements to regionalization.

The housing constraint is the most intractable and the most important to name honestly. Vermont's rental vacancy rate of 3% is well below the 5% that characterizes a healthy rental market. Half of Vermont renters are cost-burdened. Vermont has the second-highest per-capita homelessness rate in the country. Healthcare providers recruited to rural Vermont communities face a housing market that may make it impossible to find an affordable place to live near the hospital where they would work. Oliver Wyman's first imperative — build housing and fix transportation — is a healthcare workforce policy as much as a social policy. The transformation plans submitted in January 2026 reflect this: SVMC's plans to reduce locum provider costs, Mt. Ascutney's Nursing Pathways Program funding two cohorts of nursing students at \$190,000, and Grace Cottage's near-elimination of travel nurses through local retention strategies all reflect the same underlying constraint — rural Vermont cannot recruit and retain clinical staff at the volume and cost structure that fee-for-service hospital operations require.

A.8 Vermont Clinical Quality Indicators

Vermont's clinical quality performance reflects both the strengths of a high-coverage, primary care-oriented system and the equity gaps that demographic and geographic disadvantage produces. The Blueprint for Health, Vermont's statewide Patient-Centered Medical Home initiative, generated a 5.8 to 1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature — and provides the clinical infrastructure on which the AHEAD Model's primary care redesign builds.

Vermont's 11-point primary care access gap between white and BIPOC residents, the Northeast Kingdom's elevated chronic disease burden, and the rural-urban disparities in behavioral health access are the equity targets that the transformation agenda must measure against. These indicators are tracked in GMCB's annual performance reports and provide the baseline against which AHEAD Model equity requirements will be assessed. See Chapter 10 (The Equity Pillar) for the full equity analytics framework.

Sources

- AHS Hospital Tracking data (January 2026)
- AHS Transformation_Plans_Analysis.xlsx (January 2026)
- Individual hospital transformation plans (January 15–31, 2026)
- GMCB Annual Report 2024
- GMCB FY25 and FY26 Budget Orders
- Oliver Wyman Act 167 Report (September 2024)
- AHS Health Care System Transformation Report (November 2025)
- Vermont RHT Program Application (2025)
- AHEAD State Agreement — Cohort 2 (2025)
- KFF State Health Facts — Vermont
- GMCB Act 68 RBP Update (February 2026)
- Mt. Ascutney Transformational Grant January 15 Report (2026)
- WCAX / Vermont Business Magazine — GMCB FY25 budget coverage
- RAND Hospital Transparency; GMCB FY2026 data

Appendix B: Act 68 Statutory Timeline and Milestones

Act 68 of 2025 (S.126) established a comprehensive sequence of healthcare transformation milestones. This appendix consolidates the complete timeline for reference.

Date / Deadline	Statutory requirement	Primary authority	Status (April 2026\)
January 1, 2026	AHEAD Medicaid global budgets begin (Cohort 2 cooperative agreement preparation; Medicaid alignment)	DVHA / CMS	Operational — Medicaid HGB in effect
FY2026 (ongoing)	Hospital budget orders implementing \-1% commercial rate benchmark	GMCB	In effect — \$94.58M in FY2026 budget reductions ordered
FY2026 (ongoing)	\$2M Act 68 transformation grants to hospitals (HIT fund)	AHS	Launched September 2025 — all eligible hospitals submitted applications
FY2026 (ongoing)	Act 55 drug pricing cap (\$135M savings target)	GMCB / DFR	In effect — \$104.3M in FY2026 drug cap savings
Early 2026	Final hospital Care Transformation Plans approved by AHS	AHS	In progress — hospitals in draft plan phase
May 2026	BALANCE Model GLP-1 Medicaid launch (voluntary state participation)	DVHA / CMS	DVHA participation decision pending
July 1, 2026	Five new CCBHC entities certified (RHT Program target)	AHS / DMH	On track per RHT application schedule
July 2026	Medicare GLP-1 Bridge demonstration launches (\$50/month co-pay)	CMS	Federal program — Vermont Medicare beneficiaries eligible
FY2027 (October 1, 2026\)	Reference-based pricing (RBP) mandatory for commercial payers — maximum rates set by GMCB	GMCB	GMCB developing RBP methodology; vendor procurement underway
January 1, 2027	AHEAD Model Cohort 2 begins — Medicare FFS global budgets for Vermont	AHS / DVHA / GMCB / CMS	9-year model; primary care AHEAD and hospital global budgets launch

Date / Deadline	Statutory requirement	Primary authority	Status (April 2026\)
January 2027	BALANCE Model Medicare Part D coverage launches	CMS	Federal program
FY2028 (October 1, 2027\)	Global hospital budgets for non-CAH (PPS) Vermont hospitals	GMCB	Methodology under development; AHEAD alignment required
December 1, 2028	Statewide Health Care Delivery Strategic Plan delivered to Vermont Legislature	AHS \+ Health Care Delivery Advisory Committee	In planning — plan development begins 2026
FY2030 (October 1, 2029\)	Global hospital budgets for ALL Vermont hospitals including CAHs	GMCB	Final phase of global budget implementation
December 2031	BALANCE Model testing concludes	CMS	Federal program
October 1, 2032	All RHT Program funds must be spent	AHS / CMS	\$195M award; spending window 2026-2032
Every 3 years from 2028	Statewide Strategic Plan updated	AHS \+ Advisory Committee	Ongoing statutory obligation

Source: Act 68 of 2025 (Vermont); AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); CMS BALANCE Model (December 2025).

Appendix C: The 14-Hospital Centers of Excellence Assignment Framework

Oliver Wyman's Act 167 Recommendations proposed a Centers of Excellence (COE) framework organizing Vermont's 14 hospitals by specialty service concentration. This appendix presents the proposed assignments.

Hospital (HSA)	COE count	Key COE specialties assigned	Tier designation	RHT rural status
University of Vermont Medical Center (Burlington)	16	All major specialties including cardiac surgery, neurosurgery, complex oncology, neonatal intensive care, transplant, trauma	Tier 1 — Primary RSC	Non-rural (Chittenden County)
Southwestern Vermont Medical Center (Bennington)	9	Acute general surgery, cardiology, orthopedics, psychiatry (adult and adolescent), rehabilitation, dermatology, endocrinology, GI, pain management	Tier 1 — Secondary RSC	Rural
Brattleboro Memorial Hospital (Brattleboro)	6	Acute general surgery, cardiology, obstetrics/high-risk OB, psychiatry (adult), oncology outpatient, GI	Tier 1 — Secondary RSC	Rural
Rutland Regional Medical Center (Rutland)	5	Neurology, psychiatry (adult), cancer surgery, radiation therapy, orthopedics	Tier 1 — Secondary RSC	Rural
Central Vermont Medical Center (Barre)	5	Radiation therapy, neurology, cardiac, general surgery, obstetrics	Tier 1 — Secondary RSC	Rural
Northwestern Medical Center (St. Albans)	4	Cancer surgery, radiation oncology, orthopedics, cardiology	Tier 2 — Focused scope	Rural
Northeastern Vermont Regional Hospital (St. Johnsbury)	4	Cancer surgery, infusion/chemotherapy, orthopedics, general surgery	Tier 2 — Focused scope	Rural

Hospital (HSA)	COE count	Key COE specialties assigned	Tier designation	RHT rural status
Springfield Hospital (Springfield)	2	Memory care/dementia, psychiatry (adult)	Tier 2 — Focused scope	Rural
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology	Tier 2 — Focused scope	Rural
Mount Ascutney Hospital (White River Junction)	1	Rehabilitation	Tier 2 — Focused scope	Rural
Porter Medical Center (Middlebury)	TBD	Assessment ongoing — UVM network affiliation	Tier 2 — Focused scope	Rural
Grace Cottage Hospital (Townshend)	TBD	Palliative care specialty worth preserving; 19-bed Vermont's smallest hospital	Tier 3 — Transition assessment	Rural
Gifford Medical Center (Randolph)	TBD	Most financially vulnerable — comprehensive assessment needed	Tier 3 — Transition assessment	Rural
North Country Hospital (Newport)	TBD	Northeast Kingdom anchor; REH candidacy under assessment	Tier 3 — Transition assessment	Rural

Source: Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025). Note: COE designations are proposed frameworks from Oliver Wyman analysis; final designations subject to AHS and hospital transformation planning process.

Appendix D: AHEAD State Agreement — Key Terms and Provisions

Vermont's AHEAD Model State Agreement was signed January 17, 2025\. Vermont participates as a Cohort 2 state. This appendix summarizes the agreement's key terms for practitioner reference.

AHEAD provision	Vermont-specific terms
Cohort and start date	Cohort 2; model begins January 1, 2027; 9-year model through December 31, 2035
Total Cost of Care accountability	Vermont responsible for all-payer TCOC targets; Medicare FFS TCOC tracked annually; consequences for failure to meet targets after year 3
Hospital global budgets (Medicare FFS)	All Vermont hospitals receive prospective Medicare FFS global budgets beginning January 2027; methodology developed jointly with GMCB
Primary Care AHEAD (PC AHEAD)	Voluntary program for Vermont primary care practices; four payment pathway options including fully prospective models; replaces Blueprint Medicare PMPM payments
EAST Fund	Up to \$150M per year in additional Medicare funds for Vermont to invest in primary care, behavioral health, home health, long-term care, and SDOH — the transformation capital fund
Behavioral health and substance use	EAST Fund explicitly directed toward MH and SUD services; CMMI-approved \$1.2M in transformation grants from Cooperative Agreement Year 3 (2027)
Vermont GMCB regulatory independence	Agreement explicitly preserves GMCB's authority to set hospital rates; AHEAD does not supersede Vermont's state regulatory authority
Withdrawal rights	Vermont may withdraw before launch with 30-day notice; after launch with 6-month notice; unspent cooperative agreement funds must be returned to CMMI within 90 days of withdrawal
Cooperative agreement funds (pre-launch)	CMS provides cooperative agreement funds for pre-launch implementation activities; \$1.2M approved for Cooperative Agreement Year 3 (2027)
Quality performance	Vermont must meet quality performance standards; quality measures aligned with Blueprint, HEDIS, and Act 167/68 reporting requirements
CMMI evaluation	CMS conducts independent evaluation of Vermont's AHEAD performance; results inform national AHEAD expansion decisions

AHEAD provision	Vermont-specific terms
Relationship to Act 68	AHEAD provides federal vehicle for implementing Vermont's statutory global budget mandate; Act 68 provides Vermont's independent statutory authority that does not depend on AHEAD continuation

Source: Vermont AHEAD State Agreement (January 17, 2025); CMMI AHEAD Model documentation; AHS January 2025 Legislative Presentation.